Clinic Reach-Out Agent

LTV Founder Workflow Brief · Oct 19, 2026 · @anirudh

1. Startup in one line

We help independent primary care and weight-loss practices give their GLP-1 patients comprehensive care: side-effect management, nutrition and physical activity support.

- **A shared care team:** practices draw on a common pool of nutritionists and coaches they could not hire and keep on their own.
- **Billing handled:** we take on the insurance billing for this care, which is complex for a small practice to do.
- **More revenue, better care:** we believe the package can raise revenue per patient by up to 6x while giving patients better care. Large health systems already run this model; we are bringing it to independent practices.

We have spoken to some clinics and now want to pressure test the idea with more of them. Our job right now is to talk to as many independent practices as we can and get a sense check.

2. The workflow: Clinic Reach-Out Agent

An AI agent that phones independent practices to book a 30-minute call or visit with the doctor, and rewrites its own approach after every call that fails.

Why it matters now:

- A conversation with the doctor is the number one input to our customer research, and we need many of them before we commit to a segment on 25 October.
- Independent practices are very hard to reach. There are no warm connections, most have no published email address, and those that do rarely reply to email.
- The phone is the one channel left, but calls hit phone trees and front desks, and making them by hand is slow and hard to learn from.
- It recurs: every weekday through discovery, then again when we recruit design-partner practices for the build (30 October to 20 November).

3. User and trigger

- **User:** Anirudh, who owns practice outreach. He approves the list and the scripts, and he takes the meetings.

- **Trigger 1:** a new batch of practices is approved. Target: 10 per weekday, which matches the free Vapi number's limit of 10 outbound calls a day.
- **Trigger 2:** every call that does not end in a booked meeting starts the reviewer agent.
- **Trigger 3:** every booked meeting starts a calendar invite to both the doctor and Anirudh.

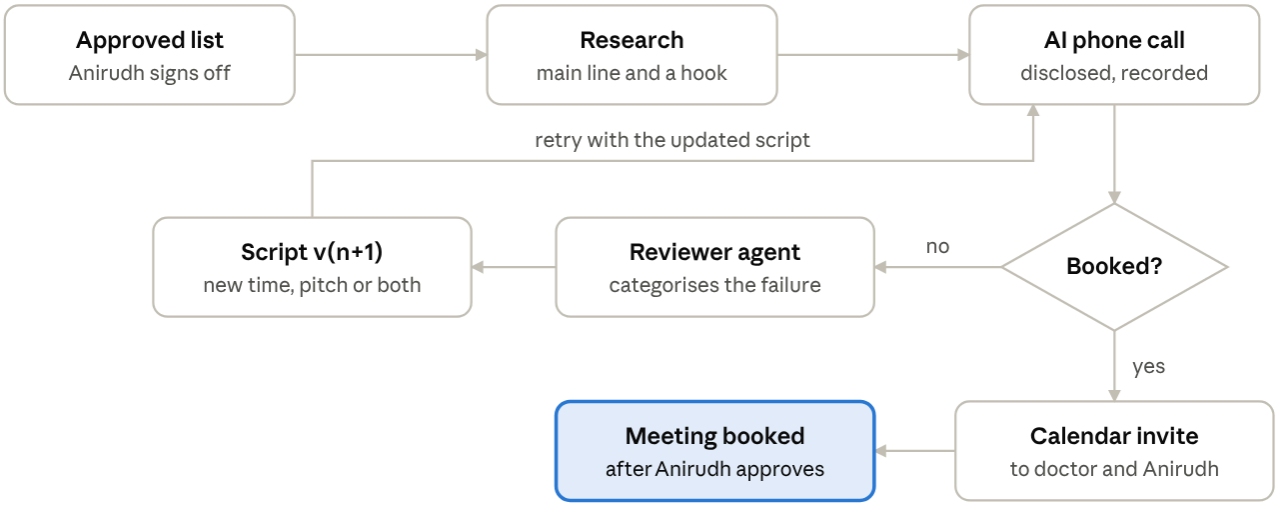
4. Input

Each call takes one practice row plus Anirudh's open slots. The row below is illustrative; the real batch of 10 Greater Boston practices is in the shared folder.

Field	Example
Practice	Example Family Practice, Brookline MA
Type	Independent family medicine, website lists medical weight management
Main phone	Published business line from the website
Lead doctor	Dr. Chen (from the practice website)
Hook	One specific line from their site, such as a new weight-management programme
Source	CMS NPI Registry plus the practice website
Open slots	Four 30-minute slots in the next 10 business days, Boston time, each marked call, visit or either, pulled from Anirudh's calendar
Script version	v1 at the start

5. Steps

Every failed call is categorised and fixes the next one



Every call is logged: transcript, recording link, outcome, failure category.

clinic reach-out loop · 8 steps, 1 decision

Every call that does not book is categorised by the reviewer, and the fix Anirudh approves (a new time, a new pitch, or both) powers the next call.

1. **Build the batch.** Pull independent family and internal medicine practices in Greater Boston from the free CMS NPI Registry. Check each website: independent, not hospital-owned, and offers GLP-1 or weight management. *Claude Code, web research.*
2. **Find the line and a hook.** Find the main phone line, the lead doctor, and one specific hook from the practice website. *Claude, web research.*
3. **Load availability.** Claude reads Anirudh's calendar and picks four open 30-minute slots, marking which suit a call and which leave time to visit. They go into the call as variables. *Anirudh's calendar.*
4. **Place the call.** Claude Code starts an outbound call to the practice's main line through the Vapi API, passing practice, doctor, hook, slots and script version. The agent discloses that it is an AI and that the call is recorded, and asks for the doctor or office manager. It pitches in under 30 seconds and asks for 30 minutes with the doctor, by phone or in person, offering two slots. Once a time is agreed, it asks where to send the calendar invite. If it cannot book, it asks when the doctor or office manager is easiest to reach. A voicemail gets a short message. *Vapi.*
5. **Log it.** Claude Code pulls the transcript, recording link, duration, outcome and failure category from the Vapi API into the tracker. *Vapi API, tracker sheet.*
6. **If booked:** Claude creates a calendar invite for the doctor and Anirudh, with call or visit details. Anirudh approves it with one click and it goes to both. *Calendar.*

7. **If not booked:** the reviewer agent reads the transcript, puts the failure in a category (table below) and finds the moment the call was lost. It changes whatever that category calls for: the call time, the pitch, the opener, the slots offered, or several at once. It writes script v(n+1) with a changelog naming the category behind each change. It cannot touch the locked lines in section 7. *Claude.*
8. **Approve and go again.** Anirudh approves or edits the changes. Claude Code pushes the new prompt and call time to Vapi, and the next call uses them, including a retry of the same practice when the category calls for one. Every 10 calls, the reviewer writes a pattern note: which categories come up most, and which fixes worked.

Failure categories. The reviewer can change more than one thing per call, and logs each change against its category so we learn which fixes work.

Failure category	How the reviewer spots it	What it changes
No answer or voicemail	No person on the line	Call time and day; voicemail message
Stuck in phone tree	Menu loops, or the call ends before a person picks up	Menu path; asks for the office manager; call time
Hung up at the AI disclosure	Call ends in the first 15 seconds	How the opener is delivered (never the disclosure itself)
Screened out by front desk	"Is this a sales call?" or "The doctor doesn't take these"	Opener; how the ask is framed; who it asks for
Pitch did not land	Reached the doctor or office manager, who said no	Value proposition, proof points, order of the pitch
Interested, no time agreed	Liked it, but no offered slot worked	Slots offered; call vs visit; time of day
Call back later	Asked to call at another time	Schedules the retry at that time; no script change
Not a fit	Hospital-owned, or no GLP-1 patients	Removes the practice; tightens the list filter

6. Output

Three things come out: a tracker, booked meetings, and a script that improves with a written reason for every change.

- **Tracker:** one row per practice with call attempts, outcome, failure category, next step, script version, transcript and recording links.
- **Booked meetings:** 30-minute calls or visits on both the doctor's and Anirudh's calendars.
- **Script history:** v1, v2 and on, each change tagged with its failure category and the call that caused it.

What great looks like, a reviewer note:

> Call 4, Example Family Practice. Category: screened out by front desk. Lost at 0:38, when the front desk asked "Is this a sales call?" and the agent went straight into the pitch. Changes: (1) the opener asks for the office manager before pitching; (2) the answer to "Is this sales?" becomes "No, we are early and not selling anything. We want Dr. Chen's honest view on whether this would help practices like yours."; (3) the retry moves to Tuesday afternoon, after the Monday rush the front desk mentioned. Reason: the front desk screens out sales calls, so reach the person who keeps the doctor's schedule first.

How we will judge it:

- Every call is logged with a transcript, the correct outcome and, if it failed, a category.
- Every reviewer change names its category and the moment the call was lost.
- The locked lines appear in every call.
- Build Day: a role-play call that fails on v1 passes on v2.
- After the first 20 real calls: the share of calls that reach a person, and the share that book, rise from v1 to the latest version. No target yet, since we have no baseline.

7. Human checkpoints and guardrails

Nothing goes out without Anirudh: the agent dials, but he approves who it calls, what it says, and every calendar invite.

1. Anirudh approves each practice batch before any call.
2. Script v1 and every new version, including changes to call times, are approved before the next call.
3. Anirudh listens to the first live calls, or reviews them within the hour.
4. Calendar invites go to the doctor only after Anirudh approves them with one click.

Locked lines (the reviewer can never edit these):

- First sentence: the caller is an AI assistant calling for Anirudh Bagla, an HBS student, about a research conversation. The FCC treats AI-generated voices as "artificial" under the TCPA, which requires identifying who is calling (source).
- Second sentence: the call is recorded for notes. Massachusetts requires every party's consent to record a call (source). If anyone objects, the agent ends politely and logs "declined recording".
- Before hanging up: Anirudh's name and a callback number.
- "Please don't call again" ends all contact with that practice.

Call rules: published main business lines only, never a doctor's mobile, since TCPA consent rules cover mobile and residential lines. Business hours only. At most two call attempts per practice unless they ask for a call back. An email address collected on a call is used only for the calendar invite. No patient information asked for or discussed, and no medical claims.

This is our reading of the rules, not legal advice. It goes on the list for the healthcare regulatory lawyer we are already looking for.

8. Definition of done (80 minutes)

Done means one full loop works: call, log, review, better script, next call.

- [ ] Vapi assistant live on a free number, with script v1 and the locked lines
- [ ] Four real open 30-minute slots from Anirudh's calendar loaded into the call
- [ ] Three calls to teammates playing front desks (friendly, gatekeeper, "just email us"), each logged with transcript, recording, outcome and failure category
- [ ] Reviewer categorises a failed call and produces script v2 with a changelog, and the next call runs on v2
- [ ] One booked outcome creates a calendar invite for both sides, sent after Anirudh approves

Tuesday evening: three real calls to approved Boston practices with Anirudh listening, all logged. Not in the 80 minutes: booking live during the call, and building batches automatically.

9. Stretch goals

- Live booking during the call: the agent checks the calendar and writes the hold through a tool call, instead of using pre-loaded slots.
- Automated batch building from the NPI Registry plus website checks.
- Test two openers across calls, and let the reviewer pick the winner.

- Phone-tree handling tuned per practice (key presses, asking for the office manager).
- Automatic re-dial at the time the reviewer sets for each failure category.
- The same loop for academic endocrinologists who refer patients into private practices.

10. Workstreams

Three teammates build in parallel; Anirudh owns the product, answers questions and approves scripts, but does not drive. The least experienced teammate drives first, and everyone drives for at least 10 minutes.

Workstream	Owns	Done when
A. Voice agent (builder)	Vapi account and free number; assistant with script v1, locked lines, call variables and voicemail message	A browser test call and a call to a real phone both work
B. Data and connections	Tracker sheet; practice batch; slots from Anirudh's calendar; Claude Code script that starts calls and pulls transcripts through the Vapi API; calendar invites	A call starts from the tracker and its transcript lands back in it
C. Reviewer, test and demo	Reviewer prompt and failure categories; three front-desk personas; runs the test calls; produces v2; records the video	v2 fixes a v1 failure on camera

11. Inputs ready to use

Shared folder: link to be added before class. No patient data is used anywhere; practice details are public business information.

- [ ] This brief, saved in the folder so Claude reads it as context
- [ ] Call script v1 (Appendix A below)
- [ ] Front-desk role-play personas (Appendix B below)
- [ ] practices.csv: 10 Greater Boston independent practices, by 8 am Tuesday
- [ ] Tracker sheet with the columns in section 6
- [ ] Anirudh's calendar access for the slot step, pulled live on the day

12. Open questions (first ten minutes)

1. Vapi or Retell? Default: Vapi, which gives each new account $10 of credit and free US numbers limited to 10 outbound calls a day (source). Retell is the backup, also with $10 of signup credit (source). Each teammate signs up so the credit stretches.
2. Which calendar: HBS Outlook or Google? Default: next 10 business days, 30-minute slots, Boston time.
3. Which callback number does the agent give?
4. Which days can Anirudh do in-person visits, and how far will he travel?
5. Real practice calls on Tuesday evening, or only once v2 passes all three role-plays?
6. Who listens to the first live call with Anirudh?

Appendix A: Call script v1

Goal: a 30-minute call or visit with the doctor. Variables in double braces are filled per call.

- **Open (locked):** "Hi, this is an AI assistant calling for Anirudh Bagla, a student at Harvard Business School. This call is recorded for our notes. Is that okay?"
- **Route:** "Could I speak with the office manager, or whoever keeps Dr. {{doctor}}'s schedule?"
- **Pitch (under 30 seconds):** "Anirudh and his co-founder help independent practices like {{practice}} give their GLP-1 patients complete care: side-effect management, nutrition and exercise support. Practices share a team of nutritionists they could not hire on their own, and the insurance billing is handled for them. We believe it can raise revenue per patient by up to six times while giving patients better care. Large health systems already run this model, and we are bringing it to independent practices. We have spoken to some clinics and would love 30 minutes with Dr. {{doctor}} to hear whether it would work here."
- **Ask for time:** "Would {{slot_1}} or {{slot_2}} work? It can be a phone call, or we can come to the practice."
- **Objections:**
  - "Is this a sales call?" Answer: "No, we are early and not selling anything yet. We want Dr. {{doctor}}'s honest view on whether this would help."
  - "The doctor is too busy." Answer: "Understood. Is there a better week, or a time of day that usually works for Dr. {{doctor}}?"
  - "Just send us an email." Answer: "Happy to. Could I hold {{slot_1}} and send the invite to that address, so Dr. {{doctor}} can accept or move it?"
- **Close (locked):** confirm the time, call or visit, and where to send the invite; give Anirudh's name and the callback number; thank them.

- **Voicemail:** "Hi, this is an AI assistant calling for Anirudh Bagla at Harvard Business School. We help independent practices give their GLP-1 patients complete care, and would love 30 minutes with Dr. {{doctor}}. Please call back at {{callback_number}}. Thank you."

Appendix B: Front-desk personas for testing

1. **Friendly:** books if offered a specific time and asks for the calendar invite.
2. **Gatekeeper:** "Is this a sales call? The doctor doesn't take these." Hangs up if the answer sounds like a pitch.
3. **Deflector:** "Just send us an email." Will not give the doctor's name unless asked a second time.

Sources

- FCC ruling that AI voices are "artificial" under the TCPA (Wilson Sonsini)
- Massachusetts call recording consent (Reporters Committee)
- Vapi free credits and free numbers (Vapi support)
- Retell AI pricing and free credit (Retell)